

Adrabetadex Document B: Commercial Payer, Medicaid, 340B, and Dual Eligible Launch Landscape

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Bottom line. Coverage will exist at launch. Friction will also exist at launch. The friction will come from combination-therapy edits, continuation criteria, site-of-care controls, specialty-pharmacy sourcing, NDC capture, and 340B / Medicaid coordination rather than from a simple yes-or-no coverage decision.

Commercial payer perspective

The commercial market already has a mature point of view on NPC therapy management. UnitedHealthcare's live commercial policies for Miplyffa and Aqneursa require genetic confirmation of NPC1 or NPC2 disease, specialist prescribing, documented benefit for reauthorization, and explicit restrictions on overlapping use. Miplyffa must be used with miglustat and not with Aqneursa. Aqneursa must be used with miglustat or after failure, contraindication, or intolerance to miglustat, and not with Miplyffa.

Comparable neurologic rare-disease policies reinforce the same pattern. Aetna's Spinraza policy requires genetic documentation, baseline functional assessment, positive response to continue treatment, and recognizes imaging-guided intrathecal administration as medically necessary. Aetna's Brineura policy requires administration by a physician knowledgeable in intraventricular administration and continuation depends on no loss or slowed loss of ambulation.

Control point	What current plans do	What that means for Adrabetadex
Eligibility proof	Require genetic confirmation and specialist involvement.	Launch access packets will need genetic proof, diagnosis narrative, and subspecialty authorship from day 1.
Continuation	Require documented positive response or slowed progression for renewal.	Reauthorization will rest on auditable clinical evidence, not general enthusiasm for the product.
Combination therapy	Block or sequence overlapping NPC therapies.	Adrabetadex will face active scrutiny if prescribers try to layer it on top of miglustat, Miplyffa, and Aqneursa without a clear sequencing rationale.
Channel control	Direct sourcing through designated specialty pharmacies and deny noncompliant drug claims.	Buy-and-bill is not guaranteed even if the drug is covered under the medical benefit.
Site of care	Treat non-hospital outpatient infusion and physician office settings as preferred; hospital outpatient requires justification.	High-cost hospital administration will need explicit medical-necessity support and site exceptions where applicable.

Medicaid behavior at launch

Medicaid is an opportunity and a workflow risk at the same time. EPSDT requires states to furnish medically necessary Medicaid-coverable services needed to correct and ameliorate conditions in beneficiaries under age 21. That makes the initial pediatric population commercially reachable once the product is approved and medically indicated. It does not convert Medicaid into an unmanaged channel. State fee-for-service programs and Medicaid managed care plans still use prior authorization, medical-necessity review, and claim-editing rules.

PAD rules make claim detail non-negotiable. CMS states that for covered outpatient physician-administered drugs, states must collect and submit utilization data and coding, and for defined categories must collect NDC data in order for payment and federal matching funds to be available. For Adrabetadex, that means a missing NDC or incomplete claim line is not a minor clean-up issue. It is a payment failure mode.

Medicaid topic	Launch reality
EPSDT	Children under 21 can access medically necessary covered services even when the state plan does not otherwise list the service in that form. States still determine medical necessity case by case.
PAD and NDC capture	NDC, utilization data, and accurate coding are mandatory for payment integrity and rebate processing.
Fee-for-service vs MCO	The state rebate framework remains in place, but authorization, claims routing, and provider burden can differ materially between FFS and managed care.
Commercial assumption of 50% Medicaid mix	A Medicaid-heavy launch magnifies the importance of hub operations, state edits, and provider billing support because errors sit in the largest payer segment.

340B economics and compliance

340B can materially improve provider acquisition economics for eligible centers of excellence. Using the published HRSA launch rule for a new covered outpatient drug, a \$39,000 WAC implies an estimated early 340B acquisition price of \$29,991 if the standard innovator rebate proxy is used. That is a \$9,009 unit-acquisition improvement before procedure cost is considered.

340B is not a free-margin program. HRSA states that duplicate discounts are prohibited and covered entities must maintain mechanisms to prevent them. Covered entities that bill Medicaid fee-for-service with 340B inventory must align carve-in status and billing identifiers with the Medicaid Exclusion File. HRSA also states that the Medicaid Exclusion File applies to Medicaid fee-for-service only and does not apply to Medicaid managed care organizations. That distinction will matter immediately if Adrabetadex is concentrated in children's hospitals and academic centers that treat both FFS and managed-care patients.

340B metric	Per administration	Annualized
Illustrative WAC	\$39,000	\$1,014,000
Estimated 340B acquisition	\$29,991	\$779,766
Estimated acquisition delta	\$9,009	\$234,234

Dual eligible behavior

Dual eligibility matters less for the initial infantile launch than for later age expansion, but the payment rule is already clear. Medicaid states that services covered by both programs are paid by Medicare first and Medicaid fills in the difference up to the state's payment limit. CMS's dual-eligibility category guidance also states that providers may not bill QMB beneficiaries for Medicare Part A or Part B deductibles, copays, or coinsurance, even though states can limit secondary payment to the lesser of the Medicare cost-sharing amount or the gap between Medicare payment and the Medicaid rate.

Dual segment	Operational effect for Adrabetadex
QMB / QMB Plus	Medicare Part B is primary for a covered outpatient clinician-administered drug claim. The beneficiary cannot be balance billed. Provider residual collection is from Medicaid only and is state-limited.
SLMB / QI	Medicaid mainly helps with premiums. Medicare remains primary for covered Part B drug claims. Secondary payment support is narrower than in full-benefit categories.

Dual segment	Operational effect for Adrabetadex
Full-benefit dual outside QMB	Medicaid can cover additional non-Medicare services, but for overlapping services Medicare still pays first and Medicaid wraps to the state's payment limit.

Numeric planning anchors

Anchor	Value	Why it matters
Annual WAC exposure at one vial every 2 weeks	\$1,014,000	This is the provider inventory at risk before any payment lag is added.
Internal GTN assumption	15%	Internal net-sales planning assumes modest leakage relative to list price.
Annual net revenue implied by 15% GTN on one-vial cadence	\$861,900	Useful as a manufacturer-planning anchor, not a provider margin figure.
Estimated 340B acquisition per vial	\$29,991	Shows why 340B hospitals can look very different from non-340B sites.
Medicare initial-sales proxy per administration	\$40,170	Concrete benchmark for provider cash-flow modeling while ASP is not yet available.

Sources

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